



EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°120/MVEBDB/11/09/2026

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HIGHLIGHTS (last 24 hours)

A total of 91 new confirmed cases, including 32 community deaths from Ebola Virus Disease (EVD), have been reported to At the end of the day on September 11, 2026, cases were reported, particularly in the provinces of North Kivu (50 cases), Ituri (33 cases), Haut-Uélé (6), Bas-Uélé (1) and Tshopo (1). In addition, 7 deaths within the treatment center were also reported. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 7,113	ACCUMULATIVE DEATH CONFIRMED 3,437	LETHALITY 48.3%	PATIENTS IN ISOLATION/CTE 855	ACCUMULATIVE HEALED 1,692	TRACKING RATES CONTACTS (Of the Day) 87.5%
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Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé and South Ubangi



62 out of 167 health zones affected (37.1%)

Summary of key points from the last 24 hours

During the course of September 11, 2026, **91 new confirmed cases** of Ebola Virus Disease (EVD), including **32 community deaths**, were recorded, suggesting a 13.8% increase in the number of confirmed cases compared to the previous day (N=80). The provinces reporting these cases are **North Kivu (50)**, **Ituri (33 cases)**, **Haut-Uélé (6)**, and **Tshopo (1)**.

and **Bas Uélé (1)**. On the other hand, **7 intra-CTE deaths** were recorded, including 3 in Ituri, 3 in North Kivu and 1 in Tshopo bringing the **total number of deaths for the day to 39**.

In addition, **21 patients** were declared cured after obtaining two negative results for Ebola Virus Disease (**11 in Ituri, 6 in Haut-Uélé and 4 in Nord-Kivu**).

Since the start of the epidemic, the DRC has accumulated **7,113 confirmed cases, including 3,437 Deaths** have been recorded in the DRC, representing a case fatality rate of **48.3 %**. The epicenter remains the Ituri province, which accounts for **78.3%** of confirmed cases reported in the 7 affected provinces, while North Kivu accounts for more than half (**54.9 %**) of new confirmed cases.

The DRC currently has **62 out of 167 health zones (37.1%)** affected and **7 Affected provinces**. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), and South Kivu (1/34). Bas-Uélé (3/11) and Sud Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 11, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	33	5571	2,533	45.5%	28/36 (77.8%)
North Kivu	50	1232	774	62.8%	16/34 (47.0%)
Haut-Uélé	6	276	115	41.7%	6/13 (46.1%)
Tshopo	1	25	10	40.0%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	1	5	3	60.0%	3/11 (27.3%)
South Ubangi	0	1	1	100.0%	1/16 (6.3%)
Total	91	7,113	3,437	48.3%	62/167 (37.1%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 11, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5571	2533	45.5%	33	6	3	9
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	179	39	21.8%	1	0		0
Boga	2	2	100.0%				0
Bunia	1550	487	31.4%	12	2		2
Damascus	36	9	25.0%				0
Drodro	15	7	46.7%				0
Fataki	78	30	38.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	127	84	66.1%	4	2		2
Lita	256	125	48.8%				0
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	46	24	52.2%	2	0		0
Mangala	310	146	47.1%				0
Mongbwalu	654	296	45.3%	5	1		1
Nia-Nia	232	117	50.4%	3	1		1
Nizi	729	273	37.4%	2	0		0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0

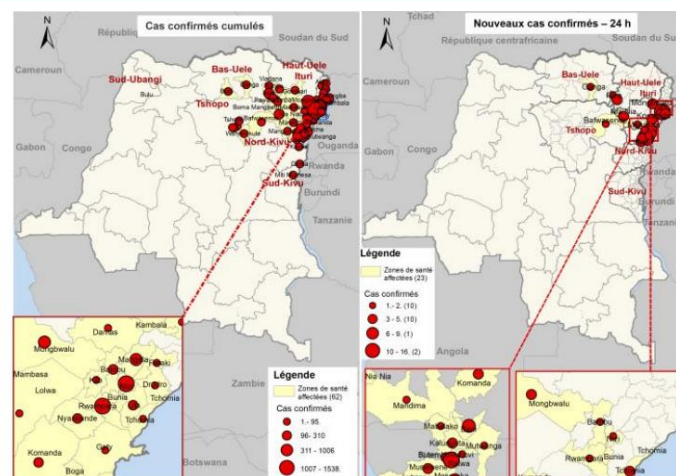
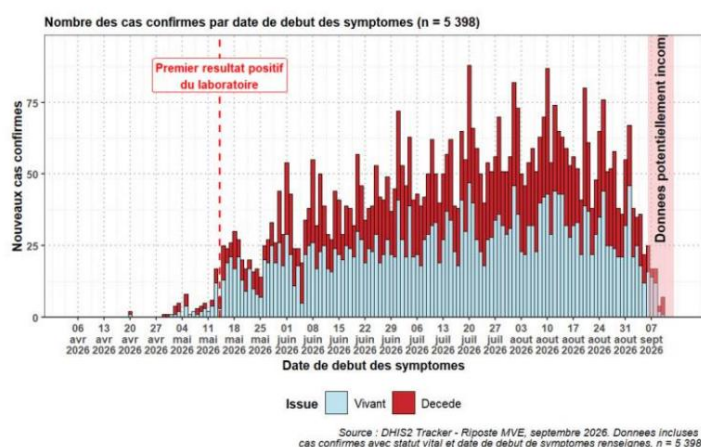
Rwampara	1007	349	34.7%	1	0		0
Tchomia	68	25	36.8%	3	0		0
To ventilate *	N / A	430	N / A			3	3
North Kivu	1232	774	62.8%	50	25	3	28
Blessed	208	156	75.0%	9	5	2	7
Good	15	6	40.0%				0
Butembo	230	152	66.1%	5	2		2
Goma	1	0	0.0%				0
Kalunguta	29	12	41.4%	5	1		1
Katwa	513	330	64.3%	14	10		10
Kayna	1	0	0.0%				
Kyondo	29	18	62.1%	0	0	1	1
Lubero	15	7	46.7%	5	3		3
Mabalako	19	9	47.4%	1	1		1
Manguredjipa	8	3	37.5%				0
Masereka	19	8	42.1%	4	2		2
Musienene	113	59	52.2%	3	1		1
Mutwanga	5	2	40.0%	2	0		0
Oicha	11	7	63.6%				0
Vuhovi	16	5	31.3%	2	0		0
Haut-Uélé	276	115	41.7%	6	1	0	1
Boma	29	14	48.3%				0
Mangbetu							
Gombari	3	1	33.3%				0
Isiro	96	41	42.7%	1	1	0	1
Pawa	57	25	43.9%	5	0	0	0
Rungu	2	1	50.0%				0
Wamba	89	33	37.1%				0
Tshopo	25	10	40.0%	1	0	1	1
Bafwasende	5	1	20.0%	1	0	1	1
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas-Uélé	5	3	60.0%	1	0	0	0
Buta	1	1	100.00%				0
Viadana	1	0	0.00%				0
Ganga	3	2	66.67%	1	0	0	0
South Ubangi	1	1	100.0%	0	0	0	0
Bulu	1	1	100.0%				0
Total	7113	3437	48.3%	91	32	7	39

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province, as of September 11, 2026

Table 1: Status of alerts notified by province, as of September 11, 2018									
DPS	Alerts Received		Total alerts	Alerts verified and		Alerts verified and		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		validated (suspected case)		invalidated (no suspected cases)			
				Living	Deceased	Living	Deceased		
Haut-Uélé	14	1	15	12	1	2	0	13	7
Bas-Uélé	21	1	22	5	1	16	0	6	0
Ituri	792	66	858	116	66	534	0	182	91
North Kivu	858	36	894	118	36	623	0	154	97
South Kivu	20	0	20	5	0	9	0	5	4
South Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo	109	5	114	7	5	102	0	12	5
Total General	1814	109	1923	263	109	1,286	0	372	204

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

In the 6 provinces out of the 7 currently affected, 1,923 alerts have been reported and 1,658 (86.2%) were verified by the end of the day on September 11, 2026. Following this process, 372 alerts were validated as suspected cases. All of these were investigated (100%), with 204 transferred to CT/CTE (Table 3).

Of the 26,763 individuals currently being monitored, 23,416 were seen, representing a daily monitoring rate of **87.5%**, which is above the 85% threshold but remains similar to the previous day. (87.6%); 89.2% in Ituri (11,959/13,401), 86.3% in Haut-Uélé (993/1,151), 85.9% in Nord-Kivu (10,156/11,823), 80.5% in Tshopo (206/256), and 77.3% in Bas-Uélé (102/132). Contact tracing around the first confirmed case is underway in Sud-Ubangi province.

1.1.3. Challenges

Contact tracing is suboptimal in 3 out of 5 provinces with ongoing contact tracing, with a significant decline in Tshopo province.

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

Four (4) alerts were notified to the various PoE/PoCs on September 11, three (3) in Ituri and one (1) in North Kivu. In Ituri, two alerts of live subjects were notified to the PoCs of Mankoko 2 (Mambasa Health Zone) and Mudzibala (Bunia Health Zone). The first was invalidated after secondary screening, and the second was isolated, validated, and then transferred to the CTE in Bunia. A body intercepted at the Zunguluka PoC (Mambasa Health Zone) was swabbed with a negative result. In North Kivu, a live alert notified to the Maboya PoC (Kalunguta Health Zone) was validated after thorough investigation and referred to the Tinge hospital.

Table 4. Status of key PoE/PoC indicators by province, as of September 11, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	93.3% (56/60)	100% (19/19)	55.5% (5/9)	100% (18/18)	54.5% (6/11)	88.9% (104/117)
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	100%
Number of people who have switched to PoE/PoC	148 004 86 590		7028	17,910	10,048	269,580
% of travelers screened	97.60% 97.60%	98.30%	99.10% 97.60%			97.70%
% of travelers who washed their hands	97.30% 97.60%	98.30%	99.30% 96.10%			97.50%
% of travelers made aware: 99.60% 97.60%	100.00%	100.00%	100.00%	99.00%		
Number of alerts notified	3	1	0	0	0	4
Number of validated live alerts (suspected cases)	1	1	0	0	0	2
Number of bodies recovered today	1	0	0	0	0	1
% of suspected cases transferred to CT/CTE	100%	100%	N / A	N / A	N / A	100%
% of lifeless bodies swabbed/secured	100%	N / A	N / A	N / A	N / A	100%
% of suspected cases investigated within 24 hours	100%	100%	N / A	N / A	N / A	100%

1.2.2. Challenges

Suboptimal operationalization of PoE/PoC coupled with low reporting completeness in Haut-Uélé (54.5%, 6/11) and Sud-Kivu (55.5%, 5/9); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri) and continuation of the strike for non-payment of service providers of three PoCs (Nzebi, Galay and Base Yedi); low encoding of alerts in DHIS2 Tracker in Ituri; in Tshopo, 18 PoCs activated out of the 30 planned; in Sud-Kivu, absence of isolation device at the PoEs of Kavimvira, Kalundu Port SNCC and Baraka.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 33 new positive results** (27 living and 6 deaths) out of 237 new samples received and analyzed (**positivity: 13.9%**).

- **North Kivu: 50 new positive results** (25 living and 25 dead) out of 237 samples analyzed (**positivity of 21.1%**).
- **Haut-Uélé: 6 new positive results** (5 living and 1 death) out of 17 samples analyzed (**positivity: 35.3%**).
- **Bas-Uélé: 1 sample** received and tested (1 live), **was found to be positive**.
- **Tshopo: 1 sample received and tested** (1 live), **was found to be positive**.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 26 of the 71 planned rings were opened (36.6%), 178 of the 212 identified households were decontaminated (84.0%), and all 11 of the 11 health facilities were decontaminated (100%). Regarding the health surveillance system (HSS), 82 alerts were recorded, of which 66 HSS tests were carried out (80.5%), and 56 bodies were swabbed.
- **In North Kivu**, 31 of the 49 planned septic tanks were opened (63.3%), 10 of the 13 identified households were decontaminated (76.9%), and 26 of the 32 health facilities were decontaminated (81.3%). Ten health facility assessments were conducted, with an average score of 32.3%. The risk of exposure assessment was rated high-risk at 38 out of 61. Regarding the health data collection (HDC), 48 alerts were recorded (24 of which were postponed from the previous day), 47 HDCs were carried out (97.9%), and 44 bodies were swabbed. There were 4 failures in Kalunguta and Vuhovi.
- **In Tshopo**, a room at the REKAPI polyclinic (Kabondo Health Zone) was decontaminated after the confirmed case from South Ubangi passed through; two infection control rings were opened in Bafwasende, with decontamination of a primary healthcare facility and six households, and the IPC/WASH kit was distributed to 10 households (100%); eight provincial laboratory staff and 114 service providers were briefed; ten community health workers were monitored and supported in five health zones; 17 local medical units were monitored, and 16 triage units were installed; 500 liters of water were supplied to Lubunga. Regarding the health data collection (HDC), no alerts were recorded, nor were any HDCs carried out in the last 24 hours.
- **In Haut-Uélé**, 6 out of 6 planned screenings were opened (100%), 14 out of 17 identified households were decontaminated (82.3%), and 4 out of 4 health facilities were decontaminated (100%), with all these actions concentrated in the health zones of Isiro and Wamba. Regarding health surveillance, 4 alerts were recorded and 4 health surveillance tests were carried out with swabs (100%) in Isiro (2) and Pawa (2).
In Bas-Uélé, 1 ring was opened out of 1 expected (100%) in Ganga; 1 EDS alert recorded and 1 body swabbed.

1.4.2. Challenges

- **In Ituri** : low ring opening (36.0%; 26/71); insufficient decontamination teams in Mambasa, Mangala and Komanda; lack of mobility in Mambasa, Bambu, Mandima and Mangala; insufficient PCI inputs in Mandima; family resistance to decontamination.
- **In North Kivu** : low household decontamination (76.9%; 10/13) and still partial opening of rings (63.3%; 31/49); need to activate, train and motivate decontamination teams in the uncovered health zones (Katwa, Biena, Masereka, Kyondo, Mabalako and Vuhovi); lack of traceability of some eligible case households; lack of mobility for the deployment of provincial IPC teams; limited promptness and completeness (68.7%; 10/16 health zones affected).
- **In Tshopo** : insufficient IPC inputs in households, schools, markets and churches; low proportion of SSEs with an IPC score above 80% (1.1%; 1/91) and incinerator of
The Makiso-Kisangani General Referral Hospital remains out of service. **In Haut-Uélé** : household kits are unavailable and sorting unit coverage is low (26.7%; 8/30). In Bas-Uélé: there is a lack of compliant waste management areas and sorting units in the 20 ESS hotspots.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 94 new admissions to the various dedicated facilities, compared to 77 discharges, including 11 recoveries. Thus, 442 patients are hospitalized out of 1007 beds, representing an occupancy rate of 43.9%; 406/421 (96%) patients received hot meals; 12/12 (100%) children are receiving LNPE/SLM at the CTE, 5/5 (100%) at the nursery, and 28/28 (100%) in the community.
- **In North Kivu**, 127 new admissions against 121 discharges recorded, including 4 recoveries; 317 patients are hospitalized, representing an overall occupancy rate of oversaturation (139.0%); 54/54 patients (100%), 1/1 accompanying person and 52/52 frontline providers received hot meals.
- **In Haut-Uélé**, there were 10 new admissions and 10 discharges (6 recovered). Thus, 65 patients are hospitalized out of 134 beds, representing an occupancy rate of 48.5%; in addition, 49 patients, 54 Accompanying persons and 28 PPLs benefited from three hot meals.
- **In Bas-Uélé**, 1 new admission against 1 non-case discharge; 6 patients remain in isolation including 1 confirmed case taken care of at the HGR of Ganga, and 9 high risk contacts are under prophylaxis in Buta and Ganga.
- **In Tshopo**, two new admissions were recorded against one discharge (death). Nine (9) patients are in isolation for 31 beds, representing an occupancy rate of 29.0%.
- **In South Kivu**, no new admissions, 16 suspected cases remain in isolation (rate occupancy rate of 64.0%).

1.5.2. Challenges

Care still partially non-standard and limited capacities: **In Ituri**, the CTE/CTs are saturated on the side of the suspects: Nizi, Mambasa, Lolwa, Kilo, Mangala and Mandima.

In North Kivu, facilities are overcrowded (139.0%), with 49% of patients receiving care outside of treatment centers.

In Haut-Uélé, the Pawa treatment center lacks hygiene facilities, and the Isiro center lacks space, water, and a constant supply of electricity. **In Tshopo**, the lack of safe blood and limited technical resources are hindering patient care. **In Bas-Uélé**,

The absence of CTE/CT and ambulance limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 2,153 people have been vaccinated to date out of 11,703 targeted TPL/PPL (18.4%), including 1,378 in Makiso-Kisangani, 312 in Kabondo, 298 in Mangobo, 122 in Tshopo and 43 in Lubunga; 3,000 doses have been deployed in the health zones, the balance being 847 doses at the zone level and 1,500 doses frozen at the Hub depot.
- **In Ituri**, the training of vaccinators and sensitizers has ended as part of the preparations for the vaccination of PPL, which is scheduled to start on September 15, 2026 in Bunia.

1.6.2. Challenges

Low uptake of vaccination among the local population and insufficient management tools **in Tshopo**, where the Bafwasende vaccination site remains inactive. **In Bas-Uélé**, the stockout of ERVEBO in Buta has led to a temporary suspension of vaccinations for local and very young people.

1.7. Continuity of Care

- **In Tshopo**, 119 cases received the experimental treatment, including 14 confirmed cases and 105 under post-exposure prophylaxis (68 treatments completed, 11 interrupted, and 26 ongoing); transfer of 3 high-risk contacts from the Ebola Treatment Center (ETC) to the transit center at Makiso-Kisangani General Referral Hospital (HGR Makiso-Kisangani), and briefing of CREC (Regional Center for Epidemiological Care) stakeholders on the operation of the ETC/CT (Transit Center/ Treatment Center). In Haut-Uélé, briefing of service providers on the use of the specific and prophylactic treatment for Ebola Virus Disease (EVD)

and obtaining, with the INRB team, informed consent from positive patients for the addition of a new molecule.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 14,833 home visits reached 50,031 people and 746 educational talks reached 13,640 people; 46 mass awareness actions reached 6,553 people, 2 community debates reached 66 people and 7 advocacy events reached 29 people; 172 community alerts were notified (including 20 deaths), 7 of the 18 feedbacks were resolved within 48 hours and the 2 recorded community incidents were resolved in Nizi and Nia-Nia.
- **In North Kivu**, 40,648 people were reached during 17,506 home visits, 662 people were reached during 58 educational talks, 767 during 6 mass communication actions and 239 during 6 briefings; 241 community alerts notified out of 450 expected (53.5%), 7 feedbacks resolved out of 12 (58.3%) and 1,010 people engaged out of 4,750 (21.2%).
- **Haut-Uélé** : 3 educational talks reached 42 people in the Pawa health zone, and actions carried out around the latest confirmed cases reached 45 people in Isiro and 31 in Pawa; 15 alerts of suspected cases and deaths were reported by community actors in Rungu, Isiro, Pawa, Boma Mangbetu, and Wamba; 3 feedback issues were resolved within 48 hours in Isiro; and preparations for the discharge of 17 survivors of Ebola virus disease are continuing in Pawa, Wamba, and Gombari.
- **In Tshopo**, five mass awareness campaigns reached 1,306 people across the five health zones of Kisangani; one educational talk was held with 28 people in Isangi; and 197 home visits were conducted, reaching 1,099 people. Two radio broadcasts and 12 public service announcements were also aired in six media outlets. Thirty-five community alerts were reported and all resolved, and three feedback sessions were conducted. In Bas-Uélé, 395 people were sensitized in schools (290 in Buta and 105 in Monga), and briefings were held for the Community Health Workers (RECO), Community Action Committees (CAC), and Community Health Coordinators (CODESA) of the Bambili Health Association.

1.8.2. Challenges

Persistent community resistance: In Tshopo, rumors of an "Ebola business," the need for handwashing facilities in schools, and the demand for the vaccine among the population fueled three infodemics, all resolved within 48 hours. A confirmed case who escaped from the Ebola Treatment Center remains at home, and the key health pillar is experiencing a shortage of posters and banners. In Ituri, there is insufficient transportation to support other key health pillars, a broken printer, and no internet connection for the key health pillar. In Haut-Uélé, there is a complete breakdown of visibility tools and communication materials in all affected health zones.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 17 people among the newly confirmed cases and 135 of the 163 confirmed cases under follow-up (83%) benefited from SMS-SPS interventions, as well as 30 of the 94 new suspected cases that day (32%) and 105 of the 159 suspected cases under follow-up (66%); 77 results were announced, including 17 positive cases; 49 PPLs were supported and 52 households were assisted; 27 children and 18 healed people followed up in the community.
- **In North Kivu**, 43 people among the new confirmed cases and 187 confirmed cases under follow-up were taken care of, as well as 128 new suspects and 176 suspects under follow-up; 106 results announced, including 43 positive ones, 56 PPL supported, 77 households supported, 11 of the 11 cured, i.e. 100% followed up; 507 people benefited from psychoeducation sessions.
- **In Tshopo**, 2 confirmed cases under monitoring, 5 new suspected cases, and 2 suspected cases under monitoring benefited from SMS-SPS interventions; 6 contacts received support at the General Referral Hospital Makiso-Kisangani, 2 chaperones at the CTE and 15 members of households and the

community supported; psychoeducation of 15 women at the REKAPI polyclinic and coaching of 2 APS from the Kabondo health zone.

1.9.2. In Haut-Uélé, 6 new confirmed cases and 24 confirmed cases under monitoring, 5 new suspected cases and 8 suspected cases under monitoring benefited from SMSPP interventions, as well as 70 caregivers at the CTE and 35 PPL; 28 laboratory results were announced, including 10 positive results, and 4 recoveries were discharged, 3 in Isiro and 1 in Wamba, without unloading kits.

1.9.3. Challenges

- **In Ituri,** there is low coverage of SMSPP interventions around the latest confirmed cases, with only 6 of the 9 notified health zones covered, and a need to strengthen collaboration with other pillars in Lita. In Haut-Uélé, there is a lack of computer equipment for the pillar, difficulties in the mobility of community health workers in Wamba and Boma Mangbetu, a lack of school and nutrition kits, and staff demotivation due to non-payment. In Tshopo, there is a lack of SMSPP providers in Wanie-Rukula, Bafwasende, and Isangi, and low partner participation in the pillar's activities.

1.10. Logistics

1.10.1. Main actions

- **In Ituri,** 15 patients were evacuated and 14 of the 33 organized transfers were carried out (42.4 %), with 3 ambulances mobilized in Bambu, Bunia and Nizi; in Haut-Uélé, 5 ambulance trips were made (Isiro 1, Pawa 2 and Boma Mangbetu 2). In North Kivu, 23 calls were received, resulting in 23 trips, of which 22 were successful (96%), and 10 ambulances were mobilized out of the 11 available (91%).
- **In Tshopo,** 2 calls received for 2 completed trips, both successful (100%), with 3 ambulances used out of the 3 available; monitoring of the construction of the Bafwasende CTE and the isolation of Madula, receipt of laboratory inputs and continuation of land procedures for the rehabilitation of the CTMI with the INSP; packing of PCI inputs received from the INSP for deployment.
- **In Bas-Uélé,** no logistical data has been transmitted in the last 24 hours.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu,** no data has been transmitted by the security pillar in the last 24 hours).

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team and a lack of financial incentive for the agents.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri,

12 response staff members (7 men and 5 women) received the PSEA briefing; 385 people (236 men and 149 women) were sensitized in the community and two partners (WHO and Mercy Corps) reported on their PSEA activities.

In Bas-Uélé, a community consultation with 65 participants was held in Koteli, and an 11-member complaints management committee, including 9 women, was established.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- Performance remains consolidated in epidemiological investigations (**100% of suspected cases investigated**) as well as a daily proportion of contact tracing (**87.5%**) similar to the previous day (87.6%) and above the threshold of 85.0%.
- In the provinces of Ituri, North Kivu, and Haut-Uélé, sustained transmission is observed, while Tshopo province shows sporadic reporting and Bas-Uélé province reported a confirmed case after more than 10 days. North Kivu province accounts for more than half (**54.9%**) of the new confirmed cases.
- reported in the last 24 hours.
- In total, **91 new confirmed cases**, including **32 community deaths and 7 intra-CTE deaths**, have been reported in the last 24 hours, bringing the national total to **7,113 confirmed cases and 3,437 deaths (case fatality rate: 48.3%)**.
- Ituri remains the epicenter of the epidemic, accounting for **78.3%** of cumulative confirmed cases, while North Kivu accounts for **54.9%** of new cases reported in the last 24 hours.
- Strengthening interventions (surveillance, care, IPC, logistics and community engagement) remain essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.
- Strengthen the provinces of the River Corridor in preparing for and responding to MVE-B.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOMBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. Jules Katusi Biselenge

Mrs. MAYELE AFUA Marie

Dr. DIUR MANANASHI Floryan

Mr. KAMUINA KEBELA John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd